C. Failed attempts at traditional IVF or conventional insemination when the quality of the ovarian stimulation was not the cause of failure.
D. Cases of IVF using preimplantation genetic testing for monogenic disorders (when a covered benefit) or structural rearrangements.
E. When using previously cryopreserved oocytes.
F. When using TESE/PESE (surgically) derived sperm
G. Recurrent molar pregnancies

When ICSI is not covered:

A. Unexplained infertility
B. Advanced maternal age
C. Low oocyte yield
D. Repeat IVF attempts after documented poor ovarian stimulation
E. Routine IVF
F. When the diagnosis is limited exclusively to teratospermia unless <2% strict morphology has been demonstrated on at least two semen analyses.
G. In the setting of PGT-A

Male Factor Infertility (High Option)

Varicocele Repair/Varicocelectomy

A. Surgical varicocelectomy is indicated in men attempting to conceive who have palpable varicocele(s), infertility, and abnormal semen parameters, except for azoospermic men.
B. Varicocelectomy is not indicated for men with non-palpable varicoceles detected solely by imaging.
C. Varicocelectomy is not indicated for men with clinical varicocele and non-obstructive azoospermia.

Sperm Retrieval

A. Surgical sperm aspiration, (Microsurgical epididymal sperm aspiration (MESA), Percutaneous epididymal sperm aspiration (PESA), Open testicular biopsy (TESE), Percutaneous testicular sperm aspiration (TEFNA) or Percutaneous testicular needle biopsy) is indicated for obstructive azoospermia in the setting of:
    1. Congenital absence of the vas deferens (carrier of cystic fibrosis gene (Jaffe, 1994), OR
    2. Infection, OR
    3. Vasectomy, OR
    4. Trauma.
B. Men with retrograde ejaculation (RE) may be treated with: